IBD medications: doses, side effects and monitoring (adults)

Crohn disease (CD) and ulcerative colitis (UC). Companion to the IBD flowsheet tab of the Hospitalist toolkit. Prepared 12 Nov 2026.

Draft reference, not an order set. Written by Claude. Doses marked **L** were checked against a current product label / SmPC on 12 Nov 2026 (risankizumab, mirikizumab, guselkumab, upadacitinib). Doses marked **G** are standard adult doses from general knowledge and have **not** been checked against a source. Confirm every dose with the Canadian product monograph, local IBD/GI service, pharmacy and BC PharmaCare Special Authority rules before prescribing. Biologic and small-molecule induction is usually a GI/IBD-service decision.

5-ASA (aminosalicylates)

Drug	Used in	Adult dose (induction → maintenance)	Key side effects	Monitoring / notes	Src
Mesalamine (Pentasa, Salofalk, Mezavant, Asacol)	UC (mainstay, mild to moderate). Little role in CD.	Oral 2.4 to 4.8 g/day (once daily dosing fine). Rectal: 1 g suppository nightly (proctitis) or 1 to 4 g enema nightly (left-sided).	Headache, nausea, diarrhea, rash. Rare: interstitial nephritis, pancreatitis, myo/pericarditis, paradoxical colitis flare.	Creatinine/eGFR at baseline, then every 6 to 12 months.	**G**
Sulfasalazine	UC; colonic CD; helps IBD arthropathy.	2 to 4 g/day in divided doses (titrate up from 500 mg bid).	GI upset, headache, rash, reversible low sperm count, orange urine. Rare: agranulocytosis, hepatitis, hemolysis (G6PD), DRESS.	CBC and LFTs every 2 to 4 weeks for 3 months, then periodically. Folic acid 1 mg daily.	**G**

Corticosteroids (induction only, never maintenance)

Drug	Used in	Adult dose (induction → maintenance)	Key side effects	Monitoring / notes	Src
Budesonide (Entocort; Cortiment/Uceris MMX)	Entocort: mild to moderate ileocecal CD. MMX: mild to moderate UC.	Entocort 9 mg daily for up to 8 weeks, taper by 3 mg every 2 weeks. MMX 9 mg daily for 8 weeks.	Fewer systemic effects than prednisone (high first-pass), but can still cause mood, glucose, adrenal suppression.	Ineffective for left colon (Entocort) or severe disease.	**G**
Prednisone	Moderate to severe flare, CD or UC.	40 mg daily (up to 1 mg/kg, max ~60 mg) until response, then taper by 5 mg/week; total course about 8 to 12 weeks.	Hyperglycemia, hypertension, insomnia, mood change, infection, osteoporosis, cataract, adrenal suppression, poor wound healing.	Glucose, K+, BP. Calcium + vitamin D; consider bone protection and PJP prophylaxis if prolonged or triple immunosuppression. 2 courses/year = steroid-dependent: escalate.	**G**
Methylprednisolone IV / hydrocortisone IV	Acute severe UC; severe CD (admitted).	Methylprednisolone 60 mg IV daily, or hydrocortisone 100 mg IV 3 to 4 times daily. Reassess on day 3 (Oxford criteria).	As prednisone; higher dose so more glucose, psychiatric and infection effects.	Exclude C. difficile and CMV; LMWH prophylaxis; daily CRP and stool count. See the hospitalist inpatient IBD guide.	**G**

Immunomodulators

Drug	Used in	Adult dose (induction → maintenance)	Key side effects	Monitoring / notes	Src
Azathioprine (Imuran)	Maintenance CD and UC; with anti-TNF to cut antibodies.	2 to 2.5 mg/kg/day oral (target dose; check TPMT/NUDT15 first). Slow onset: 8 to 12 weeks.	Leukopenia, hepatotoxicity, pancreatitis (early, idiosyncratic), nausea, infections, non-melanoma skin cancer, lymphoma (incl. HSTCL in young men with anti-TNF).	TPMT/NUDT15 before start. CBC + LFTs weekly x4, then monthly x3, then every 3 months. Avoid allopurinol unless dose cut to ~25%. Sun protection, annual skin check.	**G**
Mercaptopurine (6-MP)	As azathioprine.	1 to 1.5 mg/kg/day oral.	As azathioprine.	As azathioprine.	**G**
Methotrexate	CD (induction and maintenance); with anti-TNF. Not effective alone in UC.	Induction 25 mg SC/IM once weekly; maintenance 15 mg SC once weekly. Folic acid 1 mg daily (or 5 mg weekly).	Nausea, mouth ulcers, hepatotoxicity/fibrosis, cytopenias, pneumonitis. Teratogenic.	CBC, LFTs, creatinine monthly x3, then every 3 months. Contraception (both partners; stop 3 to 6 months before conception). Reduce or avoid if eGFR low.	**G**

Anti-TNF biologics

Drug	Used in	Adult dose (induction → maintenance)	Key side effects	Monitoring / notes	Src
Infliximab (Remicade, Inflectra, Renflexis, Avsola)	CD, UC, perianal fistula; rescue in acute severe UC.	5 mg/kg IV at weeks 0, 2, 6, then every 8 weeks. Escalate to 10 mg/kg or every 4 to 6 weeks guided by trough level. ASUC rescue: often accelerated or 10 mg/kg (GI decision).	Infusion reactions, serious infection, TB reactivation, HBV reactivation, psoriasiform rash, drug-induced lupus, demyelination, worsening heart failure, lymphoma (mainly with thiopurine).	Pre-start: IGRA/TST + CXR, HBsAg/anti-HBc/anti-HBs, HIV, HCV, VZV IgG; vaccines (no live vaccines on therapy). Loss of response: trough level + antibodies. Avoid in NYHA III/IV HF, MS.	**G**
Adalimumab (Humira and biosimilars)	CD, UC.	160 mg SC week 0, 80 mg week 2, then 40 mg every 2 weeks (may increase to 40 mg weekly or 80 mg every 2 weeks).	As infliximab, plus injection-site reactions.	As infliximab.	**G**
Golimumab (Simponi)	UC only.	200 mg SC week 0, 100 mg week 2, then 50 mg (or 100 mg if 80 kg or more) every 4 weeks.	As other anti-TNF.	As infliximab.	**G**
Certolizumab pegol (Cimzia)	CD (not approved for CD in Canada; check).	400 mg SC at weeks 0, 2, 4, then 400 mg every 4 weeks.	As other anti-TNF. Minimal placental transfer.	As infliximab.	**G**

Adalimumab: monotherapy or add an immunomodulator?

Clinical remission: no clear difference. A meta-analysis of 24 studies found adding an immunomodulator to adalimumab did not improve induction or maintenance of remission or response; a pooled analysis of CLASSIC-I, GAIN, CHARM and EXTEND found similar remission rates. **Fewer anti-drug antibodies** with combination (odds ratio 0.24). **Possibly better mucosal healing:** DIAMOND, endoscopic remission at week 26 84.2% combination vs 63.2% monotherapy; clinical remission no different; more stopped combination for side effects (16.5% vs 1.2%). AGA: evidence for adalimumab plus thiopurine over adalimumab alone is very low certainty. **Pediatric:** one trial, lower treatment failure in the adalimumab subgroup (HR 0.40), not powered for it.
Monotherapy makes most sense when infection or cancer risk matters (older patients; young males, hepatosplenic T-cell lymphoma with thiopurines; thiopurine intolerance), or when stable with good drug levels and no antibodies. On monotherapy, proactive or reactive trough and antibody checks catch immunogenicity early: 40 to 50% of anti-TNF responders lose response within 6 to 12 months.
Infliximab differs (general knowledge): combination with a thiopurine has stronger support (SONIC) and is usual. Evidence text supplied by the user 13 Nov 2026, not verified; see Aliment Pharmacol Ther 2017, doi 10.1111/apt.13838.

Anti-integrin

Drug	Used in	Adult dose (induction → maintenance)	Key side effects	Monitoring / notes	Src
Vedolizumab (Entyvio)	CD and UC (gut-selective).	300 mg IV at weeks 0, 2, 6, then every 8 weeks (every 4 weeks if loss of response). SC 108 mg every 2 weeks can replace IV from week 6.	Nasopharyngitis, headache, arthralgia, infusion reaction. Low systemic infection risk. PML not seen with vedolizumab to date (theoretical).	TB and HBV screen. Slow onset (6 to 14 weeks): often bridge with steroids. Does not treat most extraintestinal features.	**G**

IL-12/23 and IL-23 inhibitors

Drug	Used in	Adult dose (induction → maintenance)	Key side effects	Monitoring / notes	Src
Ustekinumab (Stelara and biosimilars)	CD and UC.	Single IV dose about 6 mg/kg (55 kg or less: 260 mg; 55 to 85 kg: 390 mg; over 85 kg: 520 mg), then 90 mg SC at week 8 and every 8 weeks (every 12 weeks in some).	Nasopharyngitis, headache, injection-site reactions, infection (low). Rare: PRES.	TB and HBV screen.	**G**
Risankizumab (Skyrizi)	CD and UC.	CD: 600 mg IV at weeks 0, 4, 8, then 180 or 360 mg SC (on-body injector) week 12 and every 8 weeks. UC: 1200 mg IV at weeks 0, 4, 8, then 180 or 360 mg SC every 8 weeks.	Upper respiratory infection, headache, fatigue, arthralgia, injection-site reaction. Drug-induced liver injury reported during induction.	LFTs + bilirubin at baseline and through induction (up to week 12). TB screen.	**L**
Mirikizumab (Omvoh)	UC and CD.	UC: 300 mg IV at weeks 0, 4, 8, then 200 mg SC every 4 weeks. CD: 900 mg IV at weeks 0, 4, 8, then 300 mg SC (100 + 200 mg) every 4 weeks from week 12.	Upper respiratory infection, injection-site reaction, headache, arthralgia, raised liver tests.	LFTs + bilirubin at baseline and for at least 24 weeks. TB screen.	**L**
Guselkumab (Tremfya)	UC and CD.	Induction 200 mg IV at weeks 0, 4, 8 (or, per US label, 400 mg SC at weeks 0, 4, 8). Maintenance 100 mg SC week 16 then every 8 weeks, or 200 mg SC week 12 then every 4 weeks.	Upper respiratory infection, injection-site reaction, headache, arthralgia, gastroenteritis, raised liver tests.	LFTs + bilirubin at baseline, for at least 16 weeks, then periodically. TB screen. Check Health Canada IBD indication and BC coverage.	**L**

JAK inhibitors (oral small molecules)

Drug	Used in	Adult dose (induction → maintenance)	Key side effects	Monitoring / notes	Src
Tofacitinib (Xeljanz)	UC only.	10 mg bid for 8 weeks (up to 16), then 5 mg bid (10 mg bid if loss of response, lowest effective dose).	Herpes zoster, serious infection, VTE (PE/DVT), MACE, malignancy, lymphopenia, neutropenia, lipid rise, CK rise.	CBC, LFTs, lipids at baseline and 4 to 8 weeks. Recombinant zoster vaccine (Shingrix) before start. Caution: age 50+ (65+), smokers, CV or VTE risk, cancer.	**G**
Upadacitinib (Rinvoq)	UC and CD.	UC: 45 mg daily for 8 weeks (may extend 8 more weeks), then 15 or 30 mg daily. CD: 45 mg daily for 12 weeks, then 15 or 30 mg daily. Age 65+: 15 mg maintenance. With strong CYP3A4 inhibitors: 30 mg induction, 15 mg maintenance.	As tofacitinib, plus acne. Teratogenic.	Do not start if ALC under 0.5, ANC under 1.0 or Hb under 80 g/L; hold if these occur. Contraception. Lowest effective dose; 15 mg if higher VTE/MACE/cancer risk.	**L**
Filgotinib (Jyseleca)	UC (Europe/Japan; not approved in Canada or the US).	200 mg daily (100 mg if age 75+ or eGFR 15 to 60).	As other JAK inhibitors.	Listed for completeness only.	**G**

S1P receptor modulators (UC only)

Drug	Used in	Adult dose (induction → maintenance)	Key side effects	Monitoring / notes	Src
Ozanimod (Zeposia)	UC.	Titrate: 0.23 mg daily days 1 to 4, 0.46 mg days 5 to 7, then 0.92 mg daily.	First-dose bradycardia / AV block, macular edema, raised LFTs, lymphopenia, infection (incl. zoster), hypertension. Rare PML.	Baseline ECG, LFTs, CBC, VZV IgG; eye exam if diabetes/uveitis/retinal disease. Avoid recent MI/stroke/HF, high-degree AV block. Many drug interactions (MAOIs).	**G**
Etrasimod (Velsipity)	UC.	2 mg orally once daily (no titration).	As ozanimod (bradycardia, macular edema, LFT rise, infection, headache).	As ozanimod.	**G**

Rescue therapy (acute severe UC, steroid-refractory)

Drug	Used in	Adult dose (induction → maintenance)	Key side effects	Monitoring / notes	Src
Cyclosporine IV	Acute severe UC not responding to IV steroids (alternative to infliximab).	2 mg/kg/day continuous IV infusion; switch to oral (about 4 to 5 mg/kg/day divided bid) if responding, as bridge to thiopurine or vedolizumab.	Nephrotoxicity, hypertension, seizures (low Mg or cholesterol), hypokalemia, hypomagnesemia, tremor, gum hyperplasia, opportunistic infection.	Trough levels (target per local protocol), creatinine, K+, Mg2+, cholesterol, BP. PJP prophylaxis if triple therapy. Surgery input early.	**G**

Antibiotics and supportive drugs

Drug	Used in	Adult dose (induction → maintenance)	Key side effects	Monitoring / notes	Src
Metronidazole / ciprofloxacin	Perianal CD (with drainage); pouchitis; post-op CD prophylaxis (metronidazole).	Metronidazole 500 mg bid to tid; ciprofloxacin 500 mg bid; usually 2 to 6 weeks.	Metronidazole: metallic taste, nausea, peripheral neuropathy with long use, disulfiram reaction. Cipro: tendinopathy, QT, C. difficile, aortic aneurysm.	Limit duration.	**G**
Oral vancomycin / fidaxomicin	C. difficile complicating IBD flare.	Vancomycin 125 mg PO qid x 10 days; fidaxomicin 200 mg bid x 10 days.	Few systemic effects.	Test for C. difficile in every flare before escalating immunosuppression.	**G**
IV iron (ferric derisomaltose, iron sucrose)	Iron deficiency anemia (preferred in active IBD or Hb under 100 g/L).	Ferric derisomaltose 1000 mg (20 mg/kg) single infusion; iron sucrose 200 to 300 mg per dose.	Hypersensitivity (rare), hypophosphatemia (esp. ferric carboxymaltose), skin staining.	Ferritin/TSAT 4 to 8 weeks after.	**G**
VTE prophylaxis (enoxaparin, dalteparin, heparin)	All admitted IBD flares, even with rectal bleeding.	Standard prophylactic dose (e.g. enoxaparin 40 mg SC daily).	Bleeding, HIT.	Do not hold for hematochezia unless massive.	**G**

Before any biologic, JAK or S1P: C. difficile (if flaring), TB (IGRA or TST, chest x-ray if positive or risk), hepatitis B (HBsAg, anti-HBc, anti-HBs), HCV, HIV, VZV IgG; update vaccines (inactivated anytime; live vaccines at least 4 weeks before starting and not during therapy); recombinant zoster vaccine especially before JAK; pregnancy plans; cervical screening and annual skin check if on thiopurine.

Pregnancy: avoid methotrexate, JAK inhibitors and S1P modulators. 5-ASA, thiopurines (if already on), anti-TNF, vedolizumab and ustekinumab have the most reassuring data. Involve the IBD service and obstetrics.

Label sources checked 12 Nov 2026: Skyrizi (JAMA 2024 INSPIRE/COMMAND; product label), Omvoh (DailyMed / Guideline Central label), Tremfya (US PI and UK SmPC), Rinvoq (EMA SmPC). Abbreviations: ALC absolute lymphocyte count, ANC absolute neutrophil count, ASUC acute severe ulcerative colitis, HSTCL hepatosplenic T-cell lymphoma, MACE major adverse cardiovascular events, PML progressive multifocal leukoencephalopathy.